

AND OTHER EMERGENCIES

Week 2: 5 to 11 January 2026

Data as reported by: 17:00; 11 January 2026



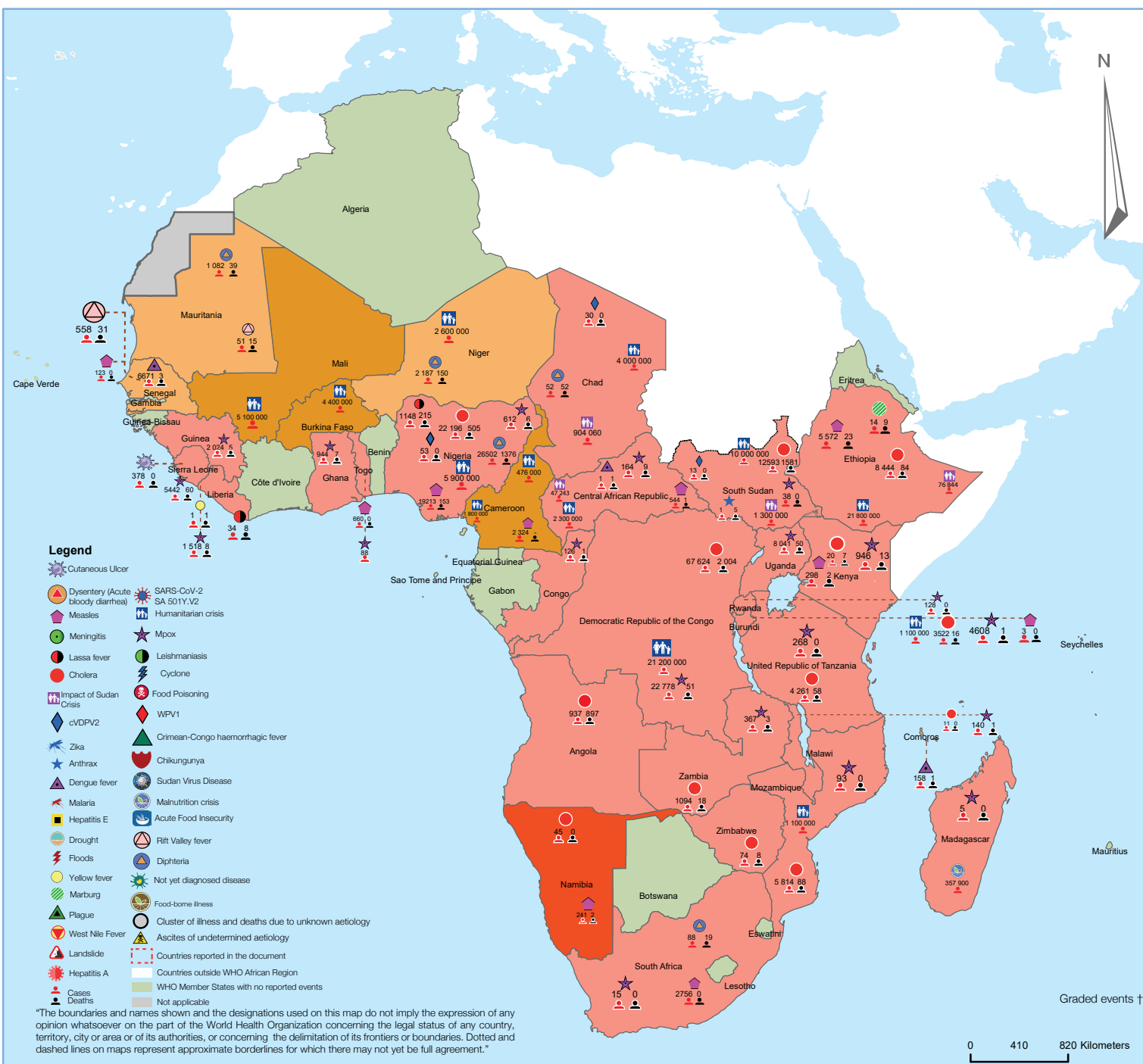
African Region

New events

Ongoing events

Outbreaks

Humanitarian crises



 Deaths  Not applicable

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Grade 3 events

Grade 2 events

Grade 1 events

Ungraded events

Protracted 3 events

Protracted 2 events

Protracted 1 events

Overview

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This Weekly Bulletin focuses on public health emergencies occurring in the WHO African Region. This week's articles cover:

- Cutaneous Ulcer in Sierra Leone
- Cholera in Namibia
- Humanitarian crisis in South Sudan

For each of these events, a brief description, followed by public health measures implemented and an interpretation of the situation, is provided.

A table is provided at the end of the bulletin with information on all new and ongoing public health events currently being monitored in the region, as well as recent events that have been controlled and closed.

Major issues and challenges include

- **Cutaneous Ulcer in Sierra Leone:** An outbreak of cutaneous ulcer in Sierra Leone largely attributed to *Haemophilus ducreyi* is unfolding amidst structural and contextual vulnerabilities, including overcrowded school and household settings, inadequate water, sanitation, and hygiene (WASH) infrastructure, and widespread barefoot exposure, which continue to facilitate transmission. High attack rates among children and students indicate sustained amplification in schools, while household clustering suggests ongoing close-contact spread. Additional challenges include incomplete laboratory confirmation for all cases, potential mixed aetiologies of cutaneous ulcers, and the need to sustain treatment supplies and behaviour change interventions over time. Without durable improvements in school WASH, hygiene practices, and community-level prevention, there remains a risk of prolonged transmission and recurrence, despite the availability of effective treatment and the absence of severe outcomes.
- **Complex Humanitarian Crisis in South Sudan:** The recent escalation of conflict from late December 2025 has sharply intensified an already fragile humanitarian situation, triggering large-scale displacement, access constraints, and direct attacks on health services in Jonglei and neighbouring states. This sudden deterioration has overwhelmed local coping capacity, disrupted health service delivery, and forced vulnerable populations into overcrowded, WASH-poor environments, amplifying the risk of cholera and other communicable disease outbreaks. Ongoing insecurity, damaged infrastructure, and limited humanitarian access remain the primary constraints to effective response, while widespread food insecurity and malnutrition continue to heighten vulnerability and undermine health outcomes.

Sierra Leone

378

Cases

0

Deaths

0.0%

CFR

Cutaneous Ulcer

EVENT DESCRIPTION

On 17 November 2025, national health authorities in Sierra Leone were notified of a cluster of cutaneous ulcer cases among pupils attending a primary school in Kayasie Village, Safroko Limba Chiefdom, Bombali District. The index case was identified as a 12-year-old boy from Small Bumbuna Village in the same district who developed symptoms on 9 October 2025.

According to the investigation, the index case reportedly had close contact with a sick goat presenting with cutaneous lesions for approximately two weeks prior to symptom onset. The animal subsequently died and was reportedly slaughtered. Retrospective community investigations identified reports of approximately 20 goats that died in Small Bumbuna Village between late September and mid-November 2025, all reportedly presenting with cutaneous lesions and similarly slaughtered. Following symptom onset, the index case reportedly had close contact with other pupils at the primary school, after which additional cases with cutaneous lesions were reported among schoolchildren.

Between 17 November 2025 and 11 January 2026, a total of 378 cases of cutaneous ulcers were reported across 27 communities in Bombali District (367 cases) and one community in Tonkolili district (11 cases). The highest number of cases were reported from Small Bumbuna (46 cases), Kagbo (42 cases), Masongbo Limba (41 cases), Masana (40 cases), and Kayasie (30 cases) communities, which together accounted for 52.6% of all reported cases. Males were slightly more affected, accounting for 57.4% (n=217) of cases. Ages ranged from 2 to 70 years (median 17 years), with the majority of cases (84.4%, n=319) occurring among individuals aged 5–20 years.

A total of 220 samples were collected for laboratory testing, including 209 human samples obtained from ulcerated lesions and 11 environmental samples collected from water sources. *Haemophilus ducreyi* was detected in 96 human samples, while all environmental samples tested negative for the pathogen.

Among cases with available clinical and exposure information, the disease presented predominantly as acute cutaneous ulcers, most frequently affecting exposed body parts, including the foot (20%), leg (17%), hand (16%), arm (15%), and face/neck (15%). Multiple lesion sites were reported in 17% of investigated cases. Lesions generally progressed rapidly to ulceration, most

commonly within 3–5 days or within one week among cases for whom progression data were available.

Among cases with symptom data recorded, itching, pain, and purulent discharge were the most frequently reported symptoms, followed by swelling and fever, indicating a predominantly localized inflammatory condition with occasional systemic manifestations. Exposure histories, available for a subset of cases, suggested potential environmental and community-level risk factors, including contact with streams or rivers prior to symptom onset, walking barefoot, and visiting schools, markets, or communities with similar cases. Nearly half of interviewed cases reported similar illnesses within the household, suggesting possible shared environmental exposures or close-contact transmission.

PUBLIC HEALTH ACTIONS

- The outbreak was officially declared and communicated by the Ministry of Health and the National Public Health Agency of Sierra Leone as an outbreak of cutaneous ulcers caused by *Haemophilus ducreyi*. An incident management system was activated to coordinate response to the event.
- Active case search and community/school-based case finding continued in affected and surrounding communities, with ongoing line listing and daily monitoring of reported cases and contacts.
- Contact follow-up was implemented for identified close contacts to support early detection and referral.
- Community-based surveillance has been strengthened through engagement of Community Health Workers and health facility staff, with emphasis on early reporting of new cases and clusters.
- Human sample collection and testing were conducted to confirm the aetiology; findings supported *H. ducreyi* as the causative agent among tested human ulcer samples.
- Environmental sampling of water sources was undertaken; environmental samples were negative for *H. ducreyi*, helping refine hypotheses regarding transmission.
- Community-level triage, wound care, and follow-up were strengthened, with case management anchored at

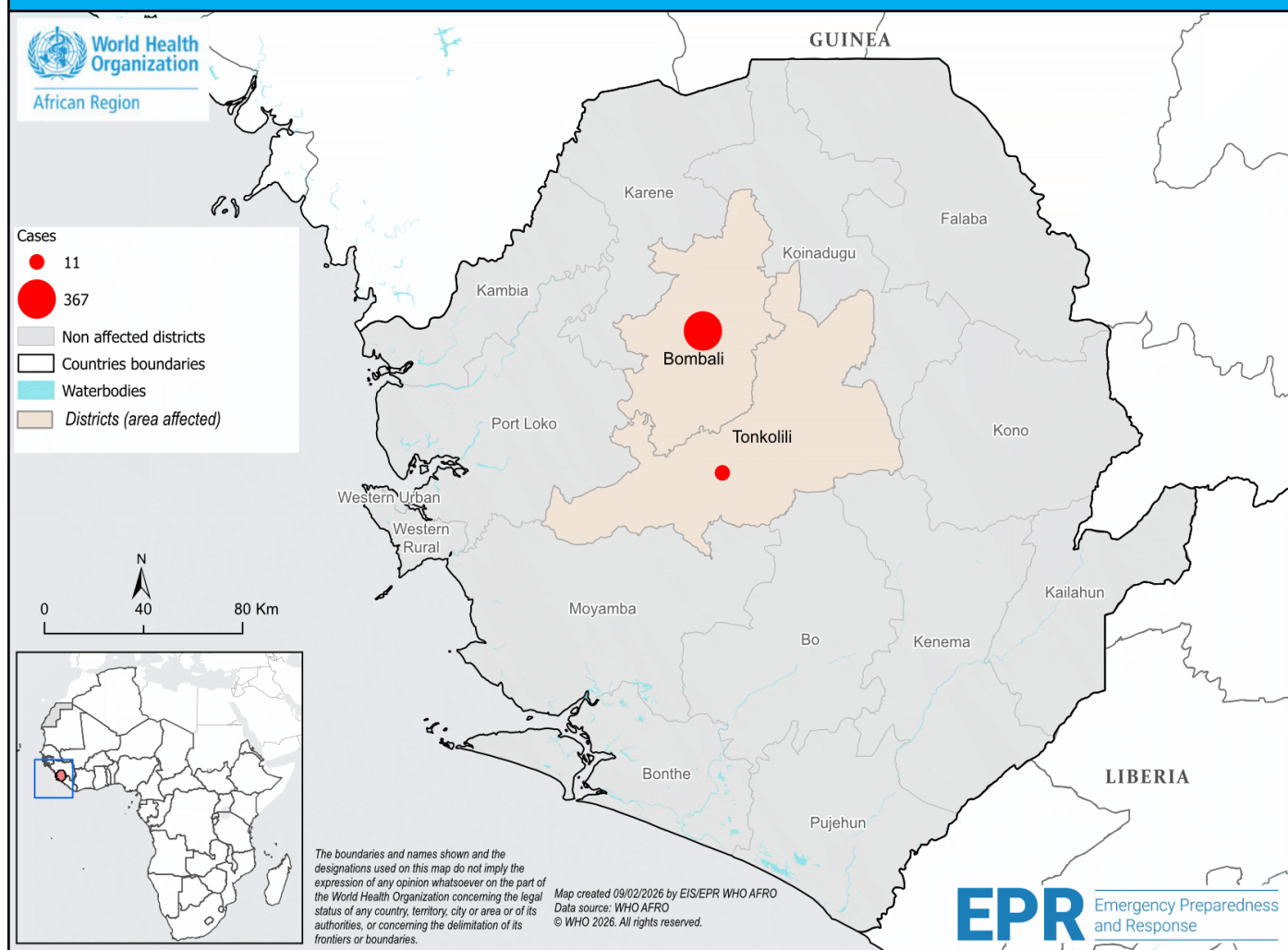
peripheral health facilities serving affected communities. Most cases were managed as stable outpatients, with continued clinical monitoring and follow-up of healing and complications.

- Azithromycin supplies were mobilized to support treatment and planned broader preventive approaches; mass drug distribution has been considered with prioritization of high-risk contacts and affected communities, aligned with NTD/yaws operational approaches where applicable.
- IPC support to facilities and communities has been implemented, including reinforcement of basic hygiene practices, safe wound care, and safe handling of potentially contaminated materials.
- Training/refresher sessions for health workers were prioritized to standardize case management, infection prevention, and referral pathways.
- School WASH and infrastructure rehabilitation was prioritized, including attention to the reported dilapidated condition of the primary school (Kayasie) and the lack of WASH facilities, to reduce exposure risks and support safer learning environments.
- School and community sensitization was conducted with teachers, pupils, CHWs, and health facility staff on early reporting, hygiene, and wound care.
- Community engagement with leaders and stakeholders emphasized preventive practices, reduced stigma, and practical steps to limit transmission. Communities have been encouraged to report suspected cases promptly via health facilities and national reporting channels (including hotline mechanisms where applicable).

SITUATION INTERPRETATION

Haemophilus ducreyi is a well-recognized cause of cutaneous ulcers in children in tropical settings. It spreads mainly through direct skin contact and minor trauma, thrives in contexts of poor hygiene and limited WASH, and commonly causes school- and household-based clusters. This event represents a context-driven outbreak of a treatable bacterial skin infection, sustained by close contact in schools and households and structural vulnerabilities rather than a point-source or zoonotic epidemic. While early animal exposures complicated initial hypotheses, the weight of evidence indicates human-to-human transmission amplified by poor WASH and school conditions. Effective control of this outbreak will depend on sustained community and school-based interventions, including early case detection and treatment, strengthened hygiene and wound care practices, improved WASH conditions, and continued risk communication and community engagement to interrupt transmission and prevent recurrence.

Geographical distribution of Cutaneous Ulcer cases, Sierra Leone, Senegal, 17 November 2025 - 11 January 2026



Namibia

45

Cases

0

Deaths

0.0%

CFR

Cholera

EVENT DESCRIPTION

On 27 November 2025, the Ministry of Health and Social Services of Namibia declared an outbreak of cholera in Grootfontein Health District in the northern Otjozondjupa region following laboratory confirmation. The presumptive index case, with symptom onset on 15 November 2025, presented to Grootfontein District Hospital on 18 November 2025. Samples sent to the Namibia Institute of Pathology tested positive for *Vibrio Cholerae* Serotype O1 on 19 November 2025.

From epidemiological week 44, 2025 (week ending 02 November 2025) to epidemiological week 2, 2026 (week ending 11 January 2026), a total of 45 cases with zero deaths (CFR 0.0%) have been reported from informal settlements in Grootfontein Health District. Of these, 24 were laboratory-confirmed by culture. Most (55.6%, n=25) cases are males. Cases range from 1 to 90 years old, with a median age of 21 years. So far, all cases reported from Namibia in this outbreak are autochthonous, with no history of travel outside of the country.

As of 11 January 2026, a total of 41 hospital admissions were

recorded, with four cases in admission under clinical care as of this date.

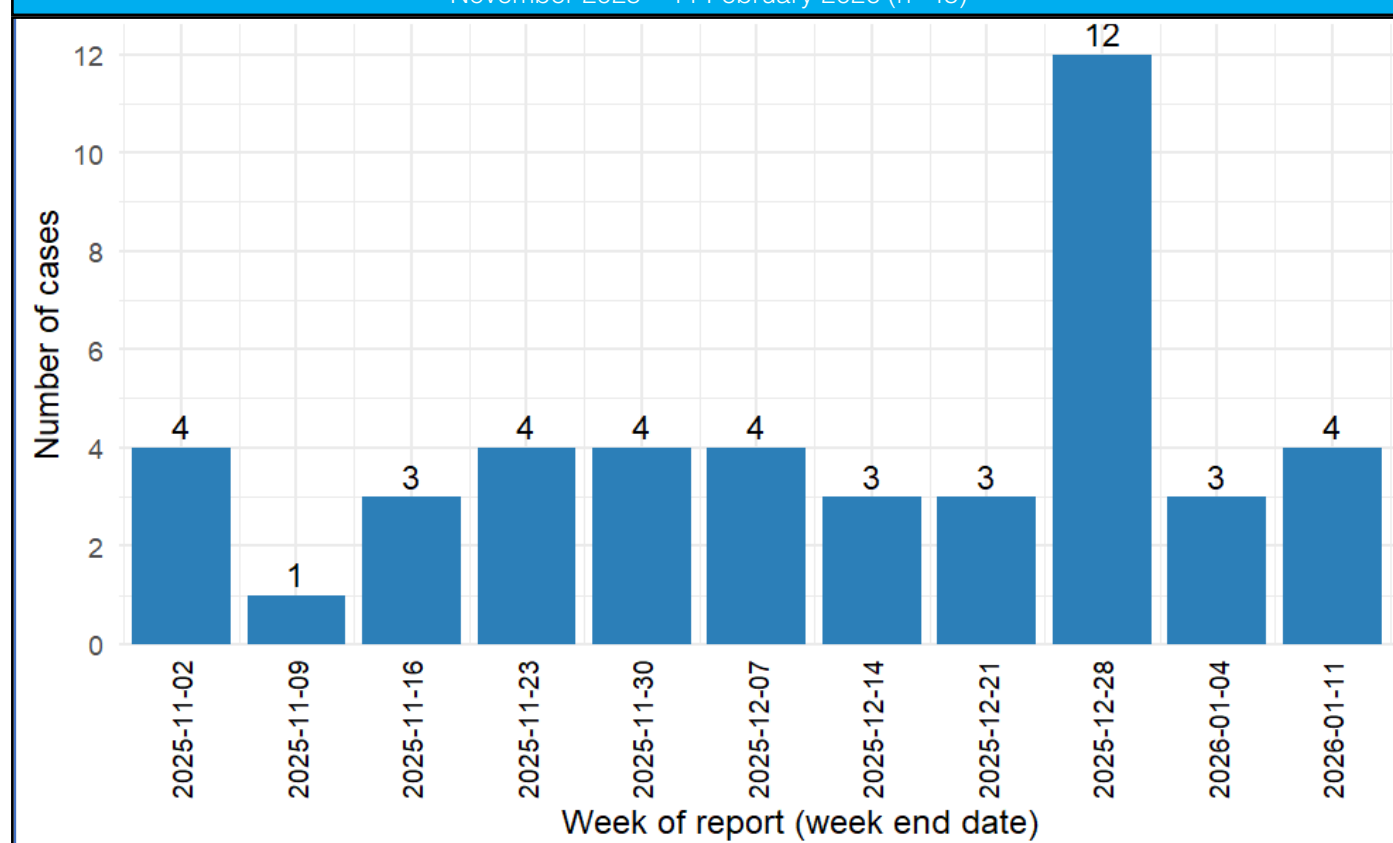
This is the first-ever documented cholera outbreak in Grootfontein District, and the second cholera outbreak recorded by Namibia in 2025. Between June and August 2025, an outbreak of cholera was reported from Opuwo District, Kunene region, with 18 suspected cases, including nine confirmed. One death was reported in this outbreak.

Despite being localized to one health district, poor WASH facilities continue to drive the outbreak and challenge response measures in the affected communities.

PUBLIC HEALTH ACTIONS

- The Ministry of Health and Social Services officially declared a second cholera outbreak in Grootfontein District on 27 November 2025 and subsequently activated an incident management system to coordinate the response. A national-level rapid response team was

Cholera cases by week of report, Grootfontein Health District, Northern Otjozondjupa Region Namibia, 02 November 2025 – 11 February 2026 (n=45)

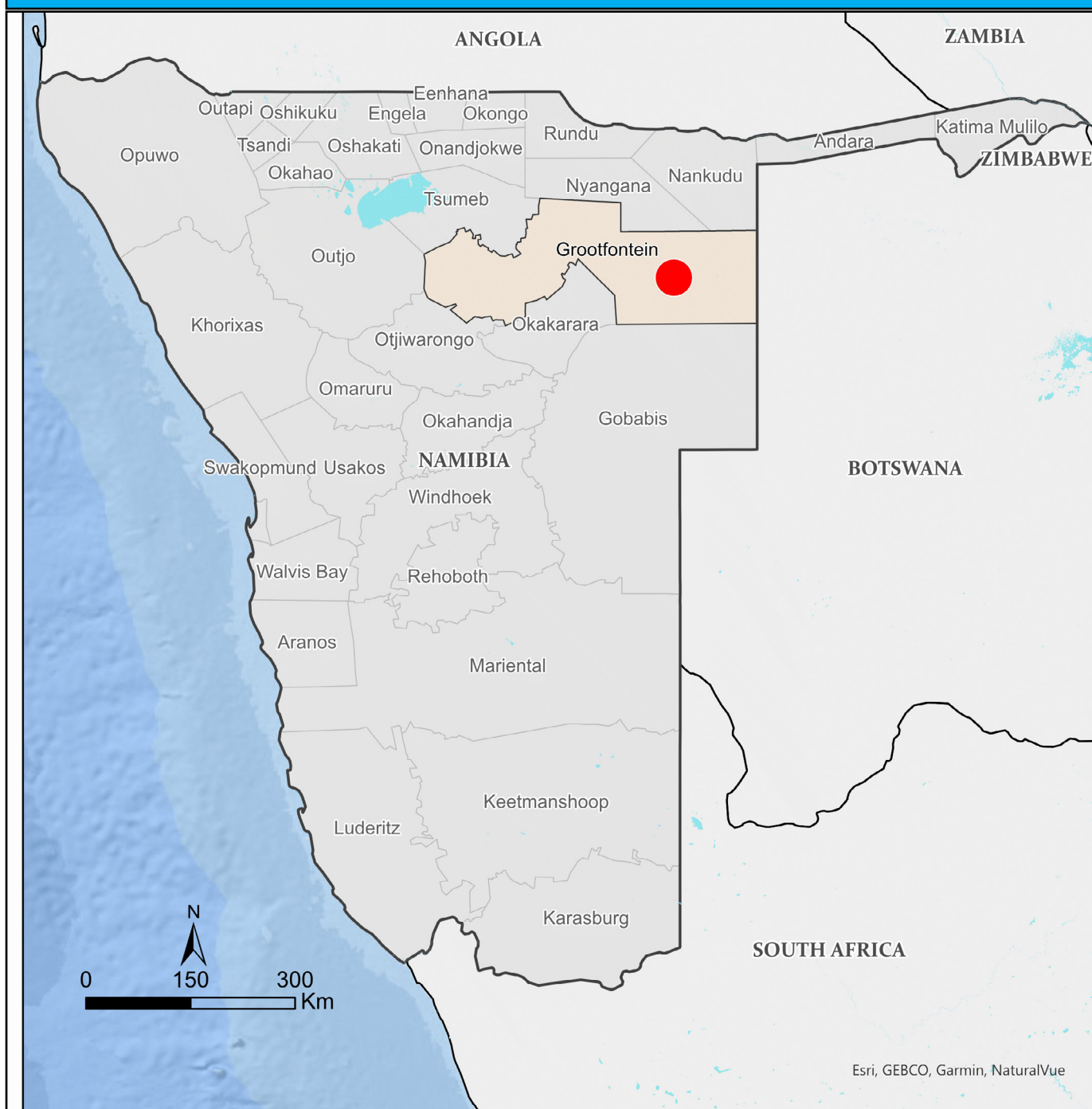


deployed to support response in the affected district.

- WHO and partners continue providing technical and financial support, including to strengthen surveillance, case management readiness, and WASH/RCCE interventions in the affected district.
- Active case finding and case investigation are ongoing across public and private health facilities and laboratories, alongside strengthened community-level detection to identify cases early and interrupt transmission.
- Case management continues in public facilities with designated areas/rooms for treatment, including provision for severe dehydration management.
- Laboratory confirmation is ongoing as part of routine surveillance. As of 11 January 2026, authorities reported 105 suspected cases, of which 24 were laboratory-confirmed by culture, 21 epidemiologically-linked, and 60 discarded due to negative test results (no *Vibrio cholerae* growth on culture and negative on rapid diagnostic test).
- Risk communication and community engagement has been intensified, including door-to-door sensitization in affected areas to promote early care-seeking, safe water handling, and hygiene practices, and to reduce misinformation and stigma.
- Local authorities have supported WASH actions including provision of potable water, distribution of safe water storage containers and water purification products, and solid waste removal in affected communities.
- Health facilities have reinforced infection prevention and control and hygiene practices to protect patients and health workers and reduce the risk of health-care–associated transmission.

SITUATION INTERPRETATION

The outbreak represents a localized but persistent cholera event driven by structural WASH vulnerabilities in informal settlements, as evidenced by autochthonous cases and geographic containment to a single health district. The absence of deaths despite substantial admissions indicates timely detection and effective clinical management, while ongoing transmission reflects continued exposure to unsafe water and sanitation conditions. Overall, the situation is currently operationally manageable, but remains vulnerable to escalation if underlying WASH conditions are not sustainably addressed.



Cases

45

 Grootfontein District (area affected)

Waterbodies

Other districts

☐ Other AFRO countries



EPR Emergency Preparedness
and Response

Map created 01/28/2026 by EIS/EPR WHO AFRO
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South Sudan

Complex Humanitarian crisis

EVENT DESCRIPTION

Since gaining independence in July 2011, South Sudan has faced a protracted and multifaceted humanitarian crisis driven by persistent conflict, political instability, economic deterioration, and recurrent climate-related shocks. These overlapping pressures, compounded by chronic insecurity and localized violence, have severely undermined livelihoods and essential services. As a result, an estimated 9.3 million people, nearly 70% of the population, require humanitarian assistance, with millions exposed to displacement, acute food insecurity, and heightened public health risks.

The situation deteriorated sharply from late December 2025 following renewed and escalating conflict, particularly in Jonglei State. Intensified fighting between government and opposition forces, including reported airstrikes, led to increased insecurity and significant constraints on humanitarian access. Active hostilities were reported in Uror, Nyirol, Akobo, and Duk counties, further isolating communities already affected by flooding, poor road infrastructure, and limited basic services.

Between 29 December 2025 and mid-January 2026, more than 180 000 people were displaced in Jonglei State, including approximately 105 000 in Uror, 57 200 in Nyirol, 9 500 in Akobo, and over 12 000 in Duk County. Population movements have extended beyond Jonglei, with over 18 000 people arriving

in Mingkaman, Lakes State, and an estimated 48 700 people displaced in Unity State due to renewed fighting. The majority of displaced individuals are women, children, and older persons, many of whom were already affected by flooding and severe food insecurity and are now living in host communities or temporary settlements with limited access to shelter, food, safe water, and sanitation.

Insecurity has severely disrupted health service delivery across affected areas. Between late December 2025 and mid-January 2026, reported incidents included the looting of four health facilities, confiscation of humanitarian assets, the relocation of 48 humanitarian workers due to insecurity, and the killing of one health worker during an attack on a primary health care facility. These disruptions have further weakened an already fragile health system at a time of increased demand.

South Sudan continues to face a high burden of concurrent disease outbreaks, including cholera, malaria, mpox, hepatitis E, measles, and anthrax. Ongoing cholera transmission persists in Duk County, Jonglei State, with rising case numbers linked to population displacement, reliance on unsafe water sources, overcrowded living conditions, and high levels of malnutrition, particularly among children under five years of age and pregnant and lactating women.



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PUBLIC HEALTH ACTIONS

- Health response coordination is being maintained at national and subnational levels, with WHO leading Health Cluster coordination, while continuously adapting operations to ongoing insecurity, access constraints, and damage to health infrastructure in affected areas.
- WHO and partners continue to support inter-agency rapid assessments, advocate for resource mobilization, and coordinate multisectoral actions to address critical gaps and sustain life-saving health interventions.
- Disease surveillance and early warning systems are being strengthened, particularly in displacement sites and host communities, through deployment of Rapid Response Teams (RRTs) and activation of cholera task forces to ensure timely detection, reporting, and response.
- Cholera preparedness and response measures are ongoing, including refurbishment and operationalization of Cholera Treatment Centres, reinforcement of case management capacity, and pre-positioning of emergency medical and IPC supplies in high-risk locations.
- Continuity of essential health services is being supported through remaining functional facilities, with emphasis on trauma care, maternal and newborn health services, routine care for chronic conditions, and emergency referral systems.
- Laboratory diagnostic capacity is being reinforced through improvements in sample collection and transportation, expansion of testing capacity where feasible, and provision of essential laboratory supplies.
- Nutrition screening and referral for children under five years of age and pregnant and lactating women are being prioritized, with linkage to available treatment services for severe acute malnutrition.
- Infection prevention and control (IPC) and WASH interventions are being strengthened across health facilities and communities to reduce the risk of cholera and other communicable disease outbreaks, particularly in overcrowded and high-risk settings.

SITUATION INTERPRETATION

The situation in South Sudan reflects a rapidly worsening humanitarian and public health emergency driven by renewed conflict layered onto pre-existing fragility, where large-scale displacement, access constraints, and climate-related shocks are converging to overwhelm already weakened health and social systems. The concentration of insecurity and displacement in Jonglei and neighbouring states is amplifying epidemic risk, particularly for cholera and other communicable diseases, through overcrowding, unsafe water and sanitation, malnutrition, and disruption of health services. While coordinated response actions are mitigating mortality and sustaining critical services, the trajectory remains highly unstable, with continued conflict and access limitations posing a significant risk of further deterioration unless protection of civilians, humanitarian access, and WASH conditions improve.

All events currently being monitored by WHO AFRO

Country	Event	Grade	Date notified to WCO	Start of reporting period	End of reporting period	Total cases	Cases Confirmed	Deaths	CFR
New Event									
Central African Republic	Dengue fever	Protracted 2	8-Jan-26	7-Jan-26	11-Jan-26	1	1	1	100.00%
On 8 January 2026, the Ministry of Health of the Central African Republic notified WHO of a confirmed dengue–yellow fever co-infection in a 28-year-old male military personnel from Bossembéle Health District. Symptom onset occurred on 25 December 2025, with subsequent progression to haemorrhagic manifestations. Samples collected on 6 January 2026 tested positive by RT-PCR for yellow fever virus and dengue virus serotype 1. The patient died on 7 January 2026. Follow-up investigations among identified contacts have not detected any additional cases.									
Ongoing Events									
Angola	Cholera	Protracted 3	7-Jan-25	8-Jan-25	10-Jan-26	36 405	937	897	2.50%
During epidemiological week 2 (as of 10 January 2026), 92 new cases, including two deaths, were reported across Angola. From 31 December 2024 to 10 January 2026, a cumulative total of 36,405 cases, including 897 deaths (CFR 2.5%) have been reported from 18 of the country's 21 provinces. Notably, 45% (n=404) of deaths occurred in the community, indicating delays in care-seeking or access to treatment. The majority of cases occurred among individuals under 30 years of age (25,007; 68%), and 54% (n=19,605) of cases were male. Luanda Province remains the most affected, accounting for 19.4% (n=7,067) of cases and 25% (n=224) of deaths.									
Burkina Faso	Humanitarian crisis (Sahel Region)	Protracted 2	1-Jan-19	1-Jan-19	8-Dec-25	4 400 000	4 400 000	-	-
Burkina Faso continues to face a protracted humanitarian crisis driven mainly by persistent insecurity, significantly affecting civilian populations. As of July 2025, more than one million internally displaced persons had returned to their areas of origin, while over 167 000 people were newly displaced between January and July 2025. The country also hosts approximately 42 000 refugees and asylum seekers. Ongoing insecurity, compounded by climate shocks and public health emergencies, continues to exacerbate vulnerabilities and disrupt access to essential services. Humanitarian partners estimate that 4.4 million people will require assistance in 2026.									
Burundi	Cholera	Protracted 3	1-Jan-23	1-Jan-25	28-Dec-25	3 522	3 522	16	0.50%
Since the previous update on 14 December 2025, 466 new cholera cases, including three deaths, were reported as of 28 December 2025. From 1 January to 28 December 2025, a cumulative total of 3,522 cholera cases with 16 deaths (CFR 0.5%) have been reported from 16 districts across the country. The burden of disease remains highly concentrated, with 92.6% (n=3,262) of cases reported from Bujumbura Nord, Cibitoke, Isare, Nyanza, Mpanda, Bujumbura South, and Ruyigi districts.									
Burundi	Complex Humanitarian crisis	Ungraded	1-Sep-23	1-Jan-24	11-Jan-26	1 100 000	-	-	-
Between 5 December 2025 and 11 January 2026, Burundi has recorded 90 790 new arrivals, following exacerbation of conflicts in eastern DRC.									
Burundi	Measles	Ungraded	18-Jan-26	1-Jan-26	11-Jan-26	11	3	0	0%
From 1 to 11 January 2026, 11 suspected measles cases were reported from six health districts, of which three were laboratory-confirmed (IgM+) in Ruyigi district (Busuma refugee camp). In 2025, over 980 suspected measles cases were recorded in Burundi.									
Burundi	Mpox	Grade 3	25-Jul-24	25-Jul-24	28-Dec-26	4 608	4 608	1	0.00%
During epidemiological week 52, nine new mpox cases were reported in Burundi. From 25 July 2024 to 28 December 2025, a cumulative total of 4,608 confirmed mpox cases with one death have been reported from 46 of the country's 49 districts. Transmission remains concentrated, with over 80% of cases reported from Bujumbura North, Bujumbura South, Bujumbura Centre, Gitega, and Isare districts. The most affected age group is 20–30 years (29%), followed by children under five years (19.8%) and adults aged 31–40 years (14.4%). The circulating virus belongs to mpox Clade Ib.									
Cameroon	Humanitarian crisis (North-West & South-West)	Protracted 2	1-Oct-16	27-Jun-18	8-Jan-26	1 800 000	-	-	-
In November 2025, the humanitarian situation in Cameroon's North-West and South-West regions remained highly volatile, characterized by persistent insecurity, population displacement, and serious protection concerns. More than 1 700 people were newly displaced, with repeated movements reported across Momo, Bui, Manyu, and Mezam divisions following armed clashes, threats, and fear of reprisals. At least 10 attacks on education were documented, alongside widespread kidnappings, intimidation of community leaders, and incidents of sexual violence, disproportionately affecting women and children. Non-state armed groups continued to impose movement restrictions and establish informal checkpoints, while intensified security operations further limited civilian mobility. Ongoing insecurity and access constraints severely disrupted livelihoods, essential services, and humanitarian operations across the region.									
Cameroon	Humanitarian crisis (Sahel Region)	Protracted 2	31-Dec-13	27-Jun-17	4-Jan-26	476 000	-	-	-
Hundreds of thousands of people have been displaced by ongoing violence along the Cameroon–Nigeria border, where recurrent insecurity, disease outbreaks, and food insecurity continue to drive humanitarian needs. In Cameroon's Far North region, communities have endured the impacts of the Lake Chad Basin crisis for more than a decade, characterized by repeated attacks, population displacement, and severe disruption of livelihoods. An estimated 476 000 people are displaced in Cameroon, contributing to a regional total of 2.9 million displaced persons across the Lake Chad Basin. Affected populations face precarious living conditions, limited access to health care, and chronic food insecurity, exacerbating vulnerabilities among displaced families and host communities.									
Cameroon	Measles	Ungraded	2-Apr-19	1-Jan-25	4-Jan-26	3 052	2 324	0	0.00%
In 2025, Cameroon reported 3 051 suspected measles cases, of which 2 323 were confirmed. Among the confirmed cases, 666 were laboratory confirmed, 1 528 were epidemiologically linked, and 129 were clinically compatible. In epidemiological week 1 of 2026, one additional clinically compatible case was reported, and 25 health districts are currently experiencing active outbreaks.									
Central African Republic	Complex Humanitarian crisis	Protracted 2	11-Dec-13	11-Dec-13	22-Dec-25	2 300 000	-	-	-

The Central African Republic continues to face a complex humanitarian crisis driven by persistent insecurity, weak governance, and regional instability. As of November 2025, an estimated 416 247 internally displaced persons remained in the country, most of whom are hosted by families, placing significant strain on already fragile host communities. Insecurity persists in the southeast and along the border with Sudan, where renewed violence has triggered further displacement. Refugee inflows from Sudan, intercommunal and transhumance-related conflicts, and limited state presence continue to undermine protection and access to basic services. According to the 2026 Humanitarian Response Plan, 2.3 million people require humanitarian assistance, with 1.3 million targeted for multisectoral support amid ongoing fragility and recurrent population movements.

Central African Republic	Impact of Sudan crisis	Grade 3	1-May-23	1-May-23	4-Jan-26	47 243	-	-	-
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As of January 2026, the Central African Republic is hosting 47 243 people fleeing the conflict in Sudan, including 40 332 refugees and 6 911 refugee returnees. Most arrivals, predominantly women and children, enter through Am-Dafock and relocate to Birao, where approximately 27 000 refugees are hosted at the Korsi settlement in Vakaga Prefecture, one of the poorest and most insecure regions of the country. Overall, 39 056 Sudanese refugees are registered, alongside 5 712 internally displaced persons living with host communities. An additional 13 351 refugees are dispersed across hard-to-reach areas, where access to assistance remains constrained by insecurity, logistical challenges, and funding shortfalls.

Central African Republic	Measles	Ungraded	-	1-Jan-25	28-Dec-25	7 322	544	1	0.00%
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In December 2025, the Central African Republic reported 559 new suspected measles cases, including eight laboratory-confirmed (IgM-positive) cases in Bangui 1, Bangui 2, and Bangui 3 health districts, and one epidemiologically linked confirmed case in Gamboula Health District. Since the start of 2025, a total of 7 322 suspected cases have been reported, including 544 confirmed cases (242 laboratory confirmed and 302 epidemiologically linked). No new confirmed cases were reported during epidemiological weeks 50 – 52.

Central African Republic	Mpox	Grade 3	3-Mar-22	31-Dec-23	28-Dec-25	164	164	9	5.50%
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Since the start of the outbreak in epidemiological week 52 of 2023, a total of 164 confirmed cases have been reported. Nine deaths have been reported among confirmed cases (CFR 5.5%). Bangui 1 and Berbérati health districts remain in active outbreak. In December 2025, clade 1b was identified for the first time in the country, in addition to clade 1a. Children under five years of age account for 22% of confirmed cases, with a male-to-female ratio of 1.6.

Chad	Diphtheria	Grade 2	1-Jan-25	1-Jan-25	21-Dec-25	4 802	52	52	1.10%
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From epidemiological week 1 to week 51 of 2025, Chad reported 4 802 suspected diphtheria cases with 52 deaths (CFR 1.1%) across seven of the country's 23 provinces. Of these, 52 were confirmed, including four laboratory confirmed cases and 48 clinically compatible cases.

Chad	Humanitarian crisis (Sahel region)	Protracted 2	11-Feb-22	1-Mar-16	8-Dec-25	4 000 000	-	-	-
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Chad continues to face a severe and protracted humanitarian crisis. An estimated 4 million people are expected to require humanitarian assistance in 2026, down from more than 7 million in 2025, but still representing a substantial proportion of the population in need. The crisis is driven by multiple, overlapping shocks, including conflict-related displacement, large influxes of refugees from neighbouring Sudan, worsening food insecurity, climate-related hazards, and limited access to essential services. Large refugee populations and ongoing internal displacement continue to place significant strain on already fragile social systems, while poor infrastructure and insecurity hinder humanitarian access and service delivery.

Chad	Impact of Sudan crisis	Grade 3	15-Apr-23	15-Apr-23	8-Dec-25	904 060	-	-	-
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Since the onset of the conflict in Sudan in April 2023, Chad has received more than 904 060 new refugees, primarily from the Darfur region, in addition to 368 421 Chadian returnees recorded by May 2025. These arrivals add to the approximately 400 000 Sudanese refugees already present in the country prior to the crisis. Over the past year, Chad has received more refugees than during the previous two decades combined and now ranks among the five largest refugee-hosting countries per capita globally, placing considerable strain on national services and host communities.

Chad	Poliomyelitis (cVDPV2)	Grade 2	18-Oct-19	1-Jan-25	4-Jan-26	30	30	0	0.00%
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Between epidemiological weeks 1 and 52 of 2025, Chad reported 26 cases of cVDPV2 and four cases of cVDPV3. The most recent case had paralysis onset on 28 October 2025.

Congo	Mpox	Grade 3	23-May-22	1-Jan-24	11-Jan-26	126	126	1	0.80%
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From 1 January 2024 to 11 January 2026, a total of 126 laboratory-confirmed mpox cases, including one death (CFR 0.8%), were reported from eight departments in the Republic of the Congo: Brazzaville, Pointe-Noire, Cuvette, Cuvette-Ouest, Sangha, Niari, Plateaux, and Likouala. Genomic sequencing identified the circulation of mpox virus clades 1a, 1b, and 11b.

Democratic Republic of the Congo	Cholera	Protracted 3	16-Jan-15	1-Jan-25	7-Dec-25	67 624	-	2 004	3.00%
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In epidemiological week 49 of 2025, the Democratic Republic of the Congo reported 1 140 suspected cholera cases, including 22 deaths (CFR 1.9%). Since the beginning of 2025, a cumulative total of 67 624 suspected cases, including 2 004 deaths (CFR 3.0%), have been reported from 237 of the 519 health zones across 21 provinces. As of week 49, 75 health zones in 13 provinces remained in active outbreak. During the preceding four weeks, 73% of reported cases were concentrated in North Kivu, Haut-Lomami, South Kivu, Tanganyika, and Maniema provinces.

Democratic Republic of the Congo	Complex Humanitarian crisis DRC	Grade 3	20-Dec-16	17-Apr-17	11-Jan-26	21 200 000	-	-	-
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Clashes have continued in eastern Democratic Republic of the Congo, including in areas around Uvira, South Kivu Province, where renewed fighting was reported during the first week of January 2026, triggering additional internal displacement and heightening protection risks for civilians. Over the past year, and as of 6 January 2026, an estimated 236,000 people have fled the DRC to neighbouring countries. Burundi is hosting more than half of the new arrivals, approximately 142,000 people, with nearly three-quarters arriving in December 2025 alone. As of 6 January, around 67,900 refugees in Burundi had been relocated from insecure border areas to the Busuma (formerly Bweru) site, where humanitarian partners are supporting the government-led response amid significant gaps in life-saving assistance.

Democratic Republic of the Congo	Mpox	Grade 3	30-Mar-19	1-Jan-25	4-Jan-26	22 778	22 778	51	0.20%
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From 24 November 2025 to 4 January 2026, the Democratic Republic of the Congo reported 447 confirmed mpox cases with zero deaths. Since the start of 2025, a cumulative total of 22 778 confirmed cases, including 51 deaths (CFR 0.2%), have been reported nationwide. The most affected provinces include Kinshasa, North Kivu, South Kivu, Bas-Uele, Tanganyika, and Tshopo.

Ethiopia	Cholera	Protracted 3	17-Sep-22	1-Jan-25	14-Dec-25	8 444	-	84	1.00%
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From 1 January to 14 December 2025, a total of 8 444 cholera cases with 84 deaths were reported in Ethiopia, with an overall case fatality rate (CFR) of 0.9%.

Ethiopia	Complex Humanitarian crisis- ETH	Protracted 2	4-Nov-20	4-Nov-20	10-Dec-25	21 800 000	-	-	-
Ethiopia continues to face multiple humanitarian emergencies driven by conflict, climate-related shocks, disease outbreaks, and regional instability. As of May 2024, an estimated 3.3 million internally displaced persons were recorded, primarily due to conflict (69%) and drought (17%), with 53% displaced for three years or more. In addition, 2.58 million internally displaced person returnees were identified, mainly in Tigray (38%), Amhara (37%), and Afar (9%). Despite the 2022 Cessation of Hostilities Agreement, insecurity persists in several regions, while recurrent droughts and floods, rising temperatures, and erratic rainfall continue to severely affect pastoralist and agro-pastoralist communities.									
Ethiopia	Impact of Sudan crisis	Grade 3	1-May-23	1-May-23	12-Oct-25	76 844	-	-	-
Following the outbreak of the conflict in Sudan on 15 April 2023, Ethiopia has experienced an influx of refugees and returnees from Sudan. As of 12 October 2025, a total of 76 844 new arrivals had been recorded, comprising 45 118 Sudanese refugees and asylum seekers, 21 018 Ethiopian returnees, and 10 708 non-Sudanese refugees and asylum seekers.									
Ethiopia	Marburg virus disease	Grade 2	11-Nov-25	14-Nov-25	28-Dec-25	14	14	9	64.30%
As of 28 December 2025, Ethiopia reported 14 confirmed cases of Marburg virus disease, including nine deaths.									
Ethiopia	Measles	Ungraded	13-Apr-17	1-Jan-25	28-Dec-25	8 845	5 572	23	0.30%
As of 28 December 2025 (Week 52), Ethiopia reported 8 845 suspected cases of measles, including 5 572 confirmed cases and 23 deaths. Active confirmed outbreaks are ongoing in two woredas from the Oromia region.									
Ghana	Mpox	Grade 3	4-Mar-25	21-Feb-25	28-Dec-25	944	944	7	0.70%
Ghana reported its first case of mpox on 4 March 2025 in a 29-year-old male from the Greater Accra Region, with symptom onset on 21 February 2025. As of 28 December 2025, a cumulative total of 944 confirmed mpox cases, including seven deaths, had been reported nationwide.									
Guinea	Mpox	Grade 3	14-Jun-25	2-Sep-24	4-Jan-26	2 024	2 024	5	0.20%
During Week 1 of 2026, Guinea reported 46 suspected mpox cases, including 33 laboratory-confirmed cases and zero deaths. From January to December 2025, 3618 suspected cases were reported, of which 2024 were confirmed. Five deaths were reported among confirmed cases.									
Kenya	Cholera	Protracted 3	1-Oct-25	17-Sep-25	28-Dec-25	296	20	7	2.40%
As of epidemiological week 52 (ending 28 December 2025), Kenya reported a total of 296 cholera cases, including 20 laboratory-confirmed cases and seven deaths (2.4%). Two new cases were reported during the final week of 2025 from Nyatike Sub-county in Migori County. The highest proportion of cases (n = 133; 45%) occurred among individuals aged 15 – 44 years. Males accounted for 195 cases (66%) of all reported cases.									
Kenya	Measles	Ungraded	29-Jun-22	1-Jan-25	28-Dec-25	895	298	2	0.20%
During epidemiological week 52 of 2025, Kenya reported 13 new measles cases, including 10 cases from Pokot South Sub-county and three from Naivasha Sub-county. Ten sub-counties are currently experiencing active outbreaks. Since the beginning of 2025, a cumulative total of 895 cases have been reported, including 298 laboratory-confirmed cases and two deaths. Males accounted for 456 cases (51%) of reported cases. Children under five years of age remain the most affected group.									
Kenya	Mpox	Grade 3	3-Aug-24	31-Jul-24	28-Dec-25	946	946	13	1.40%
In Week 52 of 2025, four new mpox cases were reported from Busia County (2), Mombasa County (1), and Nairobi County (1). Since the start of the outbreak in week 31 of 2024, a total of 946 confirmed cases, including 13 deaths have been reported.									
Liberia	Lassa Fever	Ungraded	3-Mar-22	6-Jan-25	23-Dec-25	34	34	8	23.50%
From 18 to 23 December 2025, two new confirmed Lassa fever cases were reported in Liberia. Since January 2025, a total of 215 suspected cases were reported, including 34 confirmed cases and eight deaths among confirmed cases.									
Liberia	Mpox	Grade 3	31-Aug-24	31-Aug-24	4-Jan-26	1 518	1 518	8	0.50%
From 1 December 2025 to 4 January 2026, Liberia reported 79 confirmed mpox cases, including two deaths. Since the start of the outbreak in August 2024, a total of 1 518 confirmed cases, including eight deaths, were reported. In 2025 alone, 1 455 confirmed cases and eight deaths were recorded.									
Liberia	Yellow fever	Ungraded	18-Nov-25	17-Nov-25	16-Dec-25	1	1	0	0.00%
On 17 November 2025, a 35-year-old male miner from Mano River Kongo, Porkpa District, Grand Cape Mount County, presented with symptoms compatible with yellow fever. A specimen collected the same day tested presumptive positive at the National Reference Laboratory on 18 November 2025 and was later confirmed as yellow fever at the Institut Pasteur de Dakar on 15 December 2025.									
Madagascar	Mpox	Grade 3	29-Dec-25	30-Dec-25	11-Jan-25	23	23	0	0.00%
During epidemiological week 2 (week ending on 11 January 2026), 17 new confirmed cases were reported. From 17 December 2025 to 11 January 2026, a total of 23 confirmed cases have been reported out of 131 suspected cases.									
Malawi	Cholera	Protracted 3	26-Dec-25	26-Dec-25	29-Dec-25	11	11	0	0.00%
An outbreak of cholera have been confirmed in Malawi in late December 2025 in Kasungu and Neno districts. As of 29 December 2025, a total of 11 locally acquired cases have been reported across five districts, with no deaths.									
Malawi	Mpox	Grade 3	17-Apr-25	21-Apr-25	4-Jan-26	140	140	1	0.70%
From 16 August 2024 to 23 November 2025, Malawi has confirmed 140 mpox cases, including four cross-border cases and one death. The last case was reported on 23 November 2025.									
Mali	Humanitarian crisis (Sahel region)	Protracted 2	11-Sep-17	11-Sep-17	8-Dec-25	5 100 000	-	-	-
Mali continues to face a complex humanitarian crisis driven by persistent insecurity, climate-related shocks, and structural socio-economic vulnerabilities. Violence and armed clashes, particularly in the north and centre, with increasing spillover to the south and west, continue to drive population displacement, disrupt livelihoods and education, and expose civilians to serious protection risks. The growing use of improvised explosive devices and drones has further restricted humanitarian access and civilian movement. An estimated 5.1 million people, representing approximately 20% of the population, are projected to require humanitarian assistance and protection in 2026, with food insecurity and malnutrition remaining critical concerns.									
Mauritania	Diphtheria	Grade 2	17-Sep-25	1-Jan-25	14-Dec-25	1 082	1 082	39	3.60%

Since epidemiological week 33 of 2025, Mauritania has reported an increase in diphtheria cases, predominantly affecting the Hodh Ech Chargui Region, particularly Bassiknou District, which hosts a large refugee population. Between epidemiological weeks 1 and 50 of 2025, a total of 1 082 cases and 39 deaths were reported nationwide (CFR: 3.6%). Of these, 745 cases (68.9%) with 18 deaths were reported from Hodh Ech Chargui Region.

Mauritania	Rift Valley fever	Grade 2	27-Sep-25	27-Sep-25	30-Nov-25	53	53	15	4.80%
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Since the first Rift Valley fever (RVF) case was detected on 27 September 2025 in Mauritania, a total of 53 laboratory-confirmed cases with 15 deaths (CFR 28.3%) have been reported. Transmission has affected 18 moughataas (health districts) across 12 wilayas (regions). The most recent confirmed case was reported in epidemiological week 47 of 2025.

Mozambique	Cholera	Protracted 3	30-Nov-24	28-Oct-24	21-Dec-25	5 814	5 814	88	1.50%
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As of 21 December 2025, Mozambique reported 5 814 cholera cases, including 88 deaths (CFR 1.5%), across multiple districts in Nampula, Zambezia, Tete, Manica, Cabo Delgado, and Sofala provinces. During epidemiological week 51, 257 new cases and one death were reported, representing a 13% increase compared with week 50. Eight districts remain in active outbreak, with the highest burden reported in Memba, Tete, and Moma. Individuals aged 15 years and above account for the majority of cases (83%), with no significant difference observed by sex.

Mozambique	Humanitarian crisis (Cabo Delgado)	Protracted 2	5-Sep-24	5-Sep-24	16-Jan-26	110 000	-	-	-
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In late November 2025, attacks by non-state armed groups in Nampula Province triggered large-scale displacement in northern Mozambique, with 82 691 people fleeing Memba District, primarily to Erati and onward to other districts, including parts of Cabo Delgado Province. Children accounted for approximately 67% of those displaced. By 11 December 2025, 12 580 people had returned through government-facilitated movements. The situation was further compounded by a cholera outbreak declared in Erati District on 9 December, as well as growing hostility toward health workers linked to misinformation. The violence has generated acute humanitarian needs in Erati and Memba districts, emerging as a new hotspot not currently prioritized under the 2026 Mozambique Humanitarian Response Plan.

Mozambique	Mpox	Grade 3	11-Jul-25	8-Jul-25	4-Jan-26	93	93	0	0.00%
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As of 4 January 2026, Mozambique had reported 93 laboratory-confirmed cases, with zero deaths. Confirmed cases were reported from Niassa (82), Maputo (4), Manica (3), Tete (3), and Cabo Delgado (1) provinces.

Namibia	Cholera	Protracted 3	27-Nov-25	19-Nov-25	11-Jan-26	45	24	0	0.00%
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See details in the article.

Namibia	Measles	Ungraded	14-Sep-25	18-Sep-25	4-Jan-26	863	241	2	0.20%
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The outbreak of measles in Opuwo Health District, Kunene Region, Namibia, was officially declared by the Minister of Health through a press release on 15 September 2025. As of 4 January 2026, a total of 241 confirmed cases (187 laboratory-confirmed and 54 epidemiologically linked), including two deaths, had been reported among 863 suspected cases.

Niger	Diphtheria	Grade 2	28-Aug-23	1-Jan-25	21-Dec-25	2 187	2 187	150	6.90%
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Between epidemiological weeks 1 and 51 of 2025, Niger reported a total of 2 187 diphtheria cases, including 150 deaths, resulting in a case fatality ratio of 6.9%.

Niger	Humanitarian crisis (Sahel region)	Protracted 2	1-Feb-15	1-Feb-15	8-Dec-25	2 600 000	-	-	-
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As of 30 September 2025, Niger recorded 938 000 displaced persons, representing a 5.7% increase compared with 2024, primarily in Tillabéri, Diffa, and Tahoua regions. Protection risks remain high, with 1 175 incidents affecting 2 423 victims reported between January and June 2025. Insecurity and flooding have disrupted basic services, leaving 1 097 schools non-functional and affecting 93 676 students, nearly half of whom are girls. Climate-related shocks affected approximately 542 000 people, destroying 54 891 homes and 10 400 hectares of farmland. Food insecurity and malnutrition remain severe, with an estimated 2.2 million people facing acute food insecurity and 412 000 children suffering from severe acute malnutrition. Overall, 2.6 million people are projected to require humanitarian assistance in 2026.

Nigeria	Cholera	Protracted 3	-	2-Mar-25	21-Dec-25	22 196	-	505	2.30%
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Between 1 January and 21 December 2025, Nigeria reported a total of 22 196 suspected cholera cases, including 505 deaths, resulting in a case fatality ratio of 2.4%.

Nigeria	Dengue fever	Protracted 2	11-Nov-25	13-Sep-25	17-Nov-25	75	8	6	8.00%
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In September 2025, Sokoto State, Nigeria, reported cases of a malaria-like illness unresponsive to standard treatment, prompting investigation for dengue fever. As of 17 November 2025, 75 suspected dengue cases, including eight PCR-confirmed cases, had been reported from Sokoto South and Sokoto North LGAs, with six deaths recorded among suspected cases.

Nigeria	Diphtheria	Grade 2	10-Mar-25	9-May-22	14-Dec-25	43 758	26 502	1 376	3.10%
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Between 1 January and 14 December 2025, Nigeria reported 14 857 suspected diphtheria cases, including 14 000 confirmed cases (207 laboratory-confirmed, 780 epidemiologically linked, and 13 013 clinically compatible). A total of 884 deaths were reported during the same period. Transmission remains active in five northern states: Kano, Borno, Yobe, Bauchi, and Katsina. Since 2022, over 43 000 suspected cases have been reported.

Nigeria	Humanitarian crisis (Sahel region)	Protracted 2	10-Oct-16	10-Oct-16	8-Dec-25	5 900 000	-	-	-
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Nigeria continues to face severe humanitarian needs driven by protracted conflict, insecurity, climate-related shocks, and disease outbreaks, particularly in Borno, Adamawa, and Yobe (BAY) states. Sixteen years of conflict have increasingly targeted civilians, with 4 000 deaths recorded in the first eight months of 2025, matching the total reported for all of 2023, and a renewed use of improvised explosive devices at levels last seen in 2017. Climate shocks have further compounded the crisis: floods in 2024 affected more than 5 million people, displaced approximately 1 million, and destroyed 1.3 million hectares of crops, exacerbating food insecurity amid inflation exceeding 20%. In the BAY states, an estimated 4.8 million people require emergency food and nutrition assistance, including 2 million acutely malnourished children. In 2026, approximately 5.9 million people are expected to require humanitarian assistance nationwide, with 42% of locations in Borno classified as facing life-threatening severity levels.

Nigeria	Lassa Fever	Ungraded	30-Nov-24	1-Jan-25	28-Dec-25	1 157	1 148	215	18.70%
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During epidemiological week 52, Nigeria reported 27 confirmed Lassa fever cases from 12 local government areas (LGAs) across five states. From January to December 2025, a total of 1148 confirmed and nine probable cases were reported from 107 LGAs in 22 states. Among confirmed cases, nine deaths were recorded. The majority of confirmed cases (89%) were reported from Ondo, Bauchi, Edo, and Taraba states. The most affected age group was 21–30 years, with a male-to-female ratio of 1:0.8.

Nigeria	Measles	Ungraded	1-Jan-25	1-Jan-25	30-Nov-25	19 213	19 213	153	0.80%
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In November 2025, Nigeria reported 71 suspected measles cases, including one laboratory-confirmed case. From January to November 2025, a total of 26 866 suspected cases were reported, of which 19 213 (71.5%) were confirmed (2 791 laboratory confirmed, 2 013 epidemiologically linked, and 14 409 clinically compatible). A total of 153 deaths were reported among confirmed cases (CFR 0.8%). Five states; Borno (n = 7 968), Zamfara (n = 4 779), Yobe (n = 2 076), Bauchi (n = 1 574), and Kebbi (n = 1 357), accounted for 66.1% of all suspected cases. As of November 2025, only one local government area, Eti Osa in Lagos State, remained in active measles outbreak.

Nigeria	Mpox	Grade 3	31-Jan-22	1-Jan-24	30-Nov-25	612	612	6	1.00%
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Cumulatively, from 1 January 2024 to 8 September 2025, 612 confirmed cases with six deaths were reported.

Nigeria	Poliomyelitis (cVDPV2)	Grade 2	1-Jun-18	1-Jan-25	12-Jan-26	53	53	0	0.00%
No new cVDPV2 cases were reported during the current reporting week. In 2025, Nigeria recorded a total of 53 cVDPV2 cases, with the most recent case having a paralysis onset date of 25 October 2025. The cumulative total for 2024 stands at 98 cases. By comparison, 87 cases were reported in 2023, and 48 cases in 2022.									
Senegal	Dengue fever	Protracted 2	23-Sep-25	1-Jan-25	4-Jan-26	6 671	6 671	3	0.00%
During epidemiological week 1 of 2026, three dengue cases were reported from Ndofoffane District in Kaolack Region, Senegal. In 2025, the country reported 6,668 laboratory-confirmed dengue cases, including three deaths, across 68 districts in 14 regions, with the highest burden in Kaolack, Fatick, Saint-Louis, and Dakar. The median age of cases was 23 years, and just over half (53.3%) occurred among males.									
Senegal	Measles	Ungraded	4-Jul-22	1-Jan-25	4-Jan-26	123	123	0	0.00%
From January to December 2025, Senegal reported 123 laboratory-confirmed measles cases, with males accounting for 56% of cases (n = 70). Cases were reported from 40 health districts across 12 regions, with the highest numbers recorded in Louga (n = 39), Kédougou (n = 28), Thiès (n = 13), and Kaffrine (n = 8) regions. The median age of cases was 7 years (range: 4–54 years). Among children aged 9 months to 5 years, 23 of 39 cases (59%) were unvaccinated. No cases were reported during epidemiological week 1 of 2026; however, two districts remain in active outbreak.									
Senegal	Rift Valley fever	Grade 2	26-Sep-25	13-Sep-25	4-Jan-26	558	558	31	5.60%
In 2025, Senegal reported 558 laboratory-confirmed Rift Valley fever cases, including 31 deaths, across 41 health districts in 11 regions. The highest burden was recorded in Saint-Louis Region (372 cases), followed by Matam (46 cases), Fatick (39 cases), and Kaolack (37 cases). The median age of cases was 27 years (range: 1–94 years), and males accounted for 64.7% (n=361) of cases. The most recent case had symptom onset on 14 December 2025, and no new cases have been reported in the past three weeks.									
Sierra Leone	Mpox	Grade 3	11-Jan-25	9-Jan-25	4-Dec-25	5 442	5 442	60	1.10%
From 9 January to 4 December 2025, 5 442 mpox confirmed cases with 60 deaths (CFR 1.1%) were reported from all the 16 districts of Sierra Leone.									
Sierra Leone	Cutaneous Ulcer	Ungraded	24-Nov-25	17-Nov-25	11-Jan-26	378	96	0	0.00%
See details in the article.									
South Africa	Diphtheria	Grade 2	19-Dec-24	1-Jan-24	11-Jan-26	92	90	19	20.70%
From 5 to 11 January 2026, two new laboratory-confirmed cases and one death due to toxigenic respiratory diphtheria were reported from the Western Cape Province, South Africa. Between 1 January 2024 and 11 January 2026, South Africa reported a total of 90 confirmed and two probable cases of respiratory diphtheria. The Western Cape accounted for the majority of confirmed cases (66 cases). The median age of confirmed cases was 25 years (range: 2–55 years), with 69% (62/90) occurring among individuals aged 18 years and older. The overall case fatality ratio among confirmed and probable cases was 20.7% (19/92).									
South Africa	Measles	Ungraded	16-Jun-25	19-Jun-25	28-Dec-25	2 756	2 756	0	0.00%
During epidemiological week 52 of 2025, new laboratory-confirmed measles cases were reported from Free State (n = 2), Limpopo (n = 8), Mpumalanga (n = 10), Northern Cape (n = 6), and Western Cape (n = 68). Since the beginning of 2025, a cumulative total of 2 756 laboratory-confirmed measles cases have been reported nationwide.									
South Africa	Mpox	Grade 3	30-Oct-25	1-Jan-25	11-Jan-26	17	17	0	0.00%
From 5 to 11 January 2026, two new mpox cases were notified in South Africa, both reported from KwaZulu-Natal Province. These followed the detection of three cases between epidemiological weeks 49 and 51 of 2025, after more than five consecutive weeks with no reported cases. In total, 17 laboratory-confirmed mpox cases have been reported from South Africa, originating from Gauteng (8 cases), KwaZulu-Natal (7), North West (1), and Western Cape (1) provinces.									
South Sudan	Anthrax	Ungraded	1-Aug-24	1-Jan-24	21-Dec-25	377	1	5	1.30%
In 2025, South Sudan reported a total of 216 human anthrax cases, including two deaths, all originating from Western Bahr el Ghazal State (n = 177) and Warrap State (n = 39). Since the outbreak began in 2024, a cumulative total of 377 cases have been reported, including five deaths (CFR 1.3%).									
South Sudan	Cholera	Protracted 3	11-Oct-24	28-Sep-24	28-Dec-25	97 257	12 593	1 581	1.60%
As of 28 December 2025, South Sudan has reported a total of 97 257 suspected cases since the onset of the outbreak on 28 September 2024.									
South Sudan	Complex Humanitarian crisis -SS	Protracted 3	15-Aug-16	15-Aug-16	11-Jan-26	10 000 000	-	-	-
See details in the article.									
South Sudan	Impact of Sudan crisis	Grade 3	15-Apr-23	1-May-23	11-Jan-26	1 300 000	-	-	-
The conflict in Sudan, now in its third year, continues to place severe strain on South Sudan's humanitarian system. By the end of November 2025, nearly 1.3 million refugees and returnees had entered South Sudan since April 2023, with an additional 380 000 arrivals projected by the end of 2026. This influx has overwhelmed host communities, particularly in Renk and Maban, where basic services are operating at 300 – 400% of capacity. Rising food and fuel prices, reduced humanitarian funding, and competition over land and resources have further heightened vulnerabilities and tensions. The crisis has also contributed to cross-border disease transmission, with South Sudan's ongoing cholera outbreak originating in Sudan, alongside increased risks of measles and hepatitis E.									
South Sudan	Mpox	Grade 3	7-Feb-25	7-Feb-25	4-Jan-26	38	38	0	0.00%
In 2025, South Sudan reported a total of 38 laboratory-confirmed mpox cases, with zero deaths. The most recent case was reported on 7 December 2025.									
Tanzania, United Republic of	Cholera	Protracted 3	3-Oct-23	1-Jan-25	14-Dec-25	4 261	-	58	1.40%
In 2025, as of 14 December, the United Republic of Tanzania reported a 4 261 cholera cases, including 58 deaths (CFR 1.4%). During the preceding 28 days, 21 new cases were reported.									
Tanzania, United Republic of	Mpox	Grade 3	10-Mar-25	10-Mar-25	4-Jan-26	268	268	0	0.00%
From 25 February to 31 December 2025, the United Republic of Tanzania reported 268 laboratory-confirmed mpox cases, with no deaths recorded (CFR 0%). Eighteen cases were reported during the past six weeks. The most recent case was reported on 28 December 2025.									
Togo	Measles	Ungraded	13-Oct-24	13-Oct-24	4-Jan-26	1 130	660	0	0.00%
Togo has been experiencing a measles outbreak since epidemiological week 41 of 2024. As of 4 January 2026, a total of 1130 suspected cases were reported from affected districts, including 660 laboratory-confirmed cases, with no deaths. During epidemiological week 1 of 2026, 73 new suspected cases were recorded. The median age of cases was 6 years (range 1 month–55 years).									

Togo	Mpox	Grade 3	12-May-25	2-May-25	4-Jan-26	88	88		0.00%
In 2025, Togo reported 88 laboratory-confirmed mpox cases, with the last case notified on 9 November 2025.									
Uganda	Mpox	Grade 3	26-Jul-24	29-Jul-24	4-Jan-26	8 041	8 041	50	0.60%
As of 4 January 2026 2025, Uganda has reported 8 041 confirmed cases of mpox with 50 deaths (CFR 0.6%) from 120 districts affected across the country.									
Zambia	Cholera	Protracted 3	30-Dec-24	25-Dec-24	14-Dec-25	1 094	-	18	1.60%
As of 14 December 2025, Zambia reported a cumulative total of 1 094 cholera cases, including 18 deaths (CFR 1.6%). During the preceding 28 days, 44 cases with zero deaths were recorded.									
Zambia	Mpox	Grade 3	8-Oct-24	8-Oct-24	4-Jan-26	367	367	3	0.80%
Since the outbreak began in October 2024, Zambia has reported a total of 267 laboratory-confirmed mpox cases, including three deaths, as of 4 January 2026. Of these, 265 cases and all three deaths occurred in 2025. The most recent case was reported on 14 December 2025.									

Note: There are 22 additional ongoing events not included in this table. These events will be included subsequently when additional details are available.

†Grading is an internal WHO process, based on the Emergency Response Framework. For further information, please see the Emergency Response Framework: <http://www.who.int/hac/about/erf/en/>.

Data are taken from the most recently available situation reports sent to WHO AFRO. Numbers are subject to change as the situations are dynamic.

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